

ENGLISH FOR WORK / SEPTEMBER 2026

Medical Assisting English



Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical assistants and outpatient clinic support staff.

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

after-visit summary

A document summarizing information and instructions from a clinical encounter.

allergy history

Recorded information about reported or established allergies, including details needing clinical review.

amendment

A traceable correction or addition to an existing record made through the required process.

appointment confirmation

A notice establishing a booked visit's agreed date, time, and location.

callback number

The verified telephone number to use when returning a particular call.

chaperone

A person present during an examination according to the patient's needs and the clinic's arrangements.

chief concern

The main reason a patient gives for seeking care at the current visit.

collection time

The recorded time when a specimen was obtained, distinct from receipt at the laboratory.

consent discussion

A clinician-led exchange about a proposed examination or treatment and the patient's decision.

electronic health record

The clinic's digital record of patient information, encounters, and related care documentation.

escalation

Bringing a concern to a person or pathway with the required responsibility to respond.

follow-up task

An assigned action still needed after an encounter, message, or review.

intake form

A clinic document used to gather patient information before or during a visit.

medication list

A record of medicines reported or documented for a patient, with its source and review status.

medication reconciliation

A clinician-led process of comparing medication information to identify and resolve differences.

nonprescription medicine

A medicine available without a prescription, also called an over-the-counter medicine.

onset

The time or circumstances in which a reported symptom began.

patient identifier

A detail used with the clinic's verification process to distinguish one patient from another.

preferred name

The name a patient asks staff to use when addressing them.

referral

A clinician's request for another service or professional to assess or manage a patient's care.

referral status

The recorded stage of a referral, such as sent, received, under review, or scheduled.

release of information

The provision of patient records or details to a recipient after applicable permission and identity checks.

reported symptom

A bodily experience described by the patient, not necessarily observed by staff.

requisition

A document or electronic request specifying the laboratory testing requested by a clinician.

rooming

Preparing a patient for the clinician's visit through the clinic's delegated intake activities.

specialist

A clinician whose practice focuses on a particular field of healthcare.

specimen

A sample of material from a patient submitted for examination or testing.

specimen identifier

A label or code linking a specimen to the appropriate patient and order.

teach-back

An invitation for a patient to explain their understanding so the clinician can check the explanation.

triage

Clinical assessment used to determine the urgency and appropriate response to a healthcare concern.

vital signs

Basic physiological measurements, such as temperature and pulse, recorded for clinical review.

warm transfer

A telephone handoff in which the sender introduces the caller's issue to the receiving person.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Acknowledge the expectation before checking the booking

The schedule says ___; you expected ___.

How would you describe your main concern today?

I can address you as ___ while verifying the record under ___.

A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Word combinations in this lesson

confirm the visit purpose: check what the patient expects to discuss

verify patient identifiers: establish the correct record using the clinic's checks

clarify an intake response: ask what an unclear answer means

use the preferred name: address the patient as requested without confusing record identification

Separate a report from an interpretation

The patient reports ___ since ___.

I observed ___ during ___.

The cause and ___ have not been established in this account.

Use 'reports' for the patient's experience and 'I observed' for what you directly noticed. Neither phrase establishes a diagnosis. Time words such as 'since Monday' and 'on Monday' convey different durations.

Word combinations in this lesson

record the patient's words: preserve the meaning and source of a symptom account

clarify symptom onset: ask when the patient says a change began

distinguish reported from observed: separate the patient's account from staff observations

relay a measurement query: pass uncertainty about a recorded measurement to the clinician

Ask for reconciliation, not a guessed correction

The patient reports ____; the local list still shows ____.

We do not yet have ____ from ____.

Could you review this difference with ____?

A patient report is important evidence to relay, not an instruction the assistant independently verifies. Keep source, date, and uncertainty visible. Do not infer a medicine from color or recommend changes.

Word combinations in this lesson

compare medication lists: identify differences between two stated sources

attribute a reported change: say who reported a medication change

flag an allergy-history discrepancy: draw attention to conflicting recorded allergy information

request clinician reconciliation: ask the responsible clinician to resolve medication-list differences

Clarify which event a timestamp describes

Does ____ refer to collection or receipt?

The requisition records ____; the courier sheet records ____.

Which source confirms ____?

An accurate time paired with the wrong event is misleading. Ask who recorded the time and what happened then. This lesson does not teach specimen collection, labeling, handling, acceptance, or testing procedures.

Word combinations in this lesson

match the requisition: compare the requested test details with the accompanying record

clarify the collection time: obtain the time of sampling rather than delivery

flag a label mismatch: report disagreement between identifiers for review

confirm laboratory receipt: establish that the receiving laboratory has logged the specimen

Separate referral progress from a booked visit

The referral was ____ on ____; it is now ____.

____ is our status-check date, not ____.

We still need confirmation of ____ before ____.

Sent, received, reviewed, and scheduled name distinct stages. Give the known stage and next contact without predicting the specialist's decision or timing.

Word combinations in this lesson

track a referral: check its progress through the receiving service

confirm receipt with the specialist office: establish that the request arrived

distinguish review from booking: avoid treating request assessment as an appointment

verify the clinic location: check which site is named in the appointment confirmation

Preserve the question and the patient's preference

The patient wants ___ clarified before ___.

I haven't changed or interpreted ___.

Could you return to ___ and check ___?

Identify the unclear phrase and requested conversation setting. A clarification question is not refusal of all care. Teach-back checks the explanation, not the patient's intelligence.

Word combinations in this lesson

invite a question: create space for a patient to raise an uncertainty

request a pause: communicate that the patient wants a discussion or activity stopped temporarily

clarify the after-visit summary: ask the clinician to explain unclear visit documentation

check understanding through teach-back: invite a restatement without testing or blaming the patient

Do not let a booking answer a clinical question

I can't determine ___ from ___.

The question for the nurse is ___.

I'll connect you with ___ and identify ___.

A routine appointment label does not assess a new concern. Use actual local emergency and escalation procedures for real calls. This exercise supplies no symptom criteria and teaches no triage decisions.

Word combinations in this lesson

route a clinical concern: connect the caller with the appropriate clinical response

verify the callback number: check the number for this particular return contact

make a warm transfer: introduce the concern before handing over the call

state the caller's exact request: preserve what the person wants assessed or explained

Correct the information and acknowledge its impact

My earlier message incorrectly said ___.

The confirmed information is ___.

The correction is recorded; ___ still needs to happen.

State what was wrong and what the verified record says. An amendment should remain traceable. Updating a record, reaching the patient, and finishing a follow-up task are different actions.

Word combinations in this lesson

request a record amendment: raise a specific correction for review

confirm the release recipient: establish who is to receive the requested information

trace a contact attempt: identify whether and how staff tried to reach a patient

accept follow-up ownership: explicitly take responsibility for the remaining action

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Clarifying the visit at check-in

The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

Try it again: Ask whether changing the display name also changes the verified identity record.

AI extension: build and retrieve vocabulary from this case.

02 | Rooming and factual patient reports

During rooming, patient Pat reports tiredness since Monday. The assistant observed Pat answering questions while seated but has not assessed the cause or severity. Nurse Dana is receiving the report. No measurement values or clinical conclusions are supplied.

Pat reports feeling tired since Monday. During our conversation, Pat answered questions while seated. That observation does not explain the tiredness or establish its severity. Dana, I am passing on the reported onset and what I directly observed; Pat wants to discuss the change with the clinician. I have not offered an explanation for it.

Try it again: Ask whether answering questions proves the reported concern is minor.

AI extension: build and retrieve vocabulary from this case.

03 | Medication histories with clear sources

Patient Ren says one medicine was stopped at another clinic. The local list still includes it. Ren does not have the other clinic's instruction. Nurse Dana can review the discrepancy with the clinician; the assistant cannot confirm discontinuation or change instructions.

Dana, could you review this medication-list discrepancy with the clinician? Ren reports that another clinic stopped one medicine, but our list still includes it and we do not have that instruction. Please clarify the current list with Ren. I have recorded the report as unverified and have not advised starting, stopping, or changing anything.

Try it again: Ask the assistant whether to take the medicine tonight; redirect the question to the clinician.

AI extension: build and retrieve vocabulary from this case.

04 | Laboratory paperwork and specimen queries

A requisition gives 09:20 as collection time; the courier sheet says 10:05 arrival. A colleague proposes replacing 09:20 with 10:05. Both times describe different events. No identity discrepancy or test result is stated.

Those times refer to different events. The requisition records collection at nine twenty, while the courier sheet records arrival at ten oh five. Please keep the two labels clear rather than replace one time with the other. If either source is questioned, we should ask the recording team to clarify that specific entry.

Try it again: Ask whether laboratory receipt guarantees that the specimen has been accepted for testing.

AI extension: build and retrieve vocabulary from this case.

05 | Referrals and appointment dependencies

Referral F8 was sent Monday and received Tuesday. The specialist office says it is under review, with no appointment booked. Coordinator Lia has agreed to check the status Friday. The patient believes receipt means a Friday appointment.

Referral F8 was sent Monday and received Tuesday. The specialist office is reviewing it, but no appointment has been booked. Friday is when Lia will check the status, not a visit date. We need a separate appointment confirmation before we can give you a visit time or tell you which site to attend.

Try it again: Ask whether under review means the specialist has accepted the referral.

AI extension: build and retrieve vocabulary from this case.

06 | Supporting questions during a clinical visit

Patient Jules does not understand a phrase in the after-visit summary and has not left. Dr Chen is available. The assistant cannot reinterpret the instruction. Jules wants an explanation before leaving, away from reception.

Dr Chen, Jules has a question about a phrase in the after-visit summary and would like it explained before leaving. I have not reinterpreted the instruction. Jules prefers to discuss it here rather than at reception. Could you return to the room and check the explanation with Jules before we finish the visit?

Try it again: Say you nodded to be polite but still do not understand.

AI extension: build and retrieve vocabulary from this case.

07 | Telephone concerns and clinical escalation

A caller asks the assistant to decide whether a newly reported concern can wait until next week's routine visit. No clinical details or urgency assessment are supplied. Nurse Dana is available now through the clinic's clinical-call route. The assistant cannot triage.

I can't determine from an appointment booking whether this new concern can wait until next week. Dana is available through our clinical-call route now, and I will connect you so she can assess it. I will pass on your question about timing without describing the concern as routine or offering reassurance about its importance.

Try it again: Ask for a yes-or-no answer because you do not want to repeat the question.

AI extension: build and retrieve vocabulary from this case.

08 | Records, corrections, and follow-up ownership

The assistant's message incorrectly says a results discussion is booked Tuesday. The schedule confirms Thursday at 14:00 with Dr Chen. The patient rearranged work. The assistant can correct the message and confirm the booking; no results are discussed in this exercise.

I'm sorry my message gave Tuesday for your results discussion. The confirmed appointment is Thursday at two with Dr Chen. I understand you changed your work plans because of my message. I'll correct that message in the record and check that you have the Thursday details; I cannot interpret the results during this scheduling call.

Try it again: Ask whether the wrong date says anything about the clinical importance of the results.

AI extension: build and retrieve vocabulary from this case.

Use the language responsibly

Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

U.S. Bureau of Labor Statistics: Medical Assistants

<https://www.bls.gov/ooh/healthcare/medical-assistants.htm>

American Association of Medical Assistants: Medical Assisting

<https://www.aama-ntl.org/medical-assisting>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Build vocabulary and collocations

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Assisting English

Professional audience: Medical assistants and outpatient clinic support staff

Scope: Every patient, symptom report, date, measurement context, and clinic arrangement is fictional language practice. This is not clinical, diagnostic, triage, medication, specimen-collection, privacy-law, or treatment training. Duties depend on local rules, training, delegation, and supervision. Qualified clinicians assess urgency and make care decisions; use the workplace's actual emergency and escalation procedures, not these scripts, for real concerns.

Lesson 01: Clarifying the visit at check-in

Original fictional case: The schedule says annual review. Patient Alex expected a visit about a recent concern. The assistant cannot decide whether the appointment type is suitable. Nurse Dana can clarify the visit plan. Alex prefers Alex, while the verified record says Alexandra.

Language workshop: Acknowledge the expectation before checking the booking

Communication goal: Describe a visit mismatch without dismissing the patient's reason for attending.

Language guidance: A booking label and the patient's chief concern serve different purposes. Ask what the patient expected, preserve their wording, and leave decisions about the visit's clinical suitability to the team.

Useful frames (complete the gaps with case facts): The schedule says ____; you expected _____. / How would you describe your main concern today? / I can address you as _____ while verifying the record under _____.

Vocabulary: chief concern: The main reason a patient gives for seeking care at the current visit. / intake

form: A clinic document used to gather patient information before or during a visit. / patient identifier: A

detail used with the clinic's verification process to distinguish one patient from another. / preferred name:

The name a patient asks staff to use when addressing them.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Alex: explain the different expectation and ask staff to use your preferred name.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Ask whether changing the display name also changes the verified identity record.

Model for comparison AFTER my attempt, not a response to give me first: Alex, the schedule describes an annual review, but you expected to discuss a recent concern. I will tell Dana about that difference so she can clarify the visit plan with you. I can use Alex when addressing you while keeping the verified record linked to Alexandra. I haven't confirmed a change to the appointment type.

END REFERENCE

TASK: Retrieve occupational vocabulary

1. Select four terms from the reference. Explain each in plain English, give two natural collocations and quote or create a short workplace example. Label invented examples. Contrast each term with a plausible confusable term; acknowledge regional variation.
2. Present one role-labelled exchange with one missing word or phrase and four choices A-D. Exactly one choice must fit the stated facts and grammar. Stop and wait for my answer before revealing it.
3. Explain all four options, then invite me to read the completed exchange aloud. Offer a sentence frame for optional spoken practice, never an open-ended writing assignment.
4. Repeat for the other three terms, one question at a time. Vary the correct letter and the situation; preserve the reference facts. If a choice is ambiguous, replace the item.
5. Finish with two new four-choice collocation questions for later review, with a clearly separate answer key. Do not claim to schedule a reminder.

END PROMPT